

Title

Organizational and Implementation Determinants of Hepatitis C Treatment Success and Satisfaction in Opioid Treatment Program Settings

Abstract

Understanding which operational and staffing configurations optimize outcomes is critical for dissemination. We employed mixed-methods analysis examining organizational variables (site liaison presence, case manager availability, provider workload, participant volume) and implementation characteristics affecting hepatitis C cure rates and patient satisfaction. Sites with on-site care coordinator presence demonstrated higher cure rates (84% vs. 62% at sites without coordinator, $p < 0.001$) and satisfaction (7.6 vs. 5.3, $p < 0.001$). Lower provider-to-patient ratios (defined as fewer than 25 active cases per provider) were associated with superior outcomes (cure 85% vs. 70%; $p < 0.001$). In the referral-model sites, higher case manager education, more intensive site-based engagement activities, and university affiliation were associated with better outcomes. Telemedicine sites required fewer optimization variables to achieve high success, while standard-referral sites required more intensive supportive infrastructure. These findings suggest implementation pathways suited to different OTP capacity levels.